CAP: advanced salivary gland cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: CAP

Active medicines: Cyclophosphamide + Doxorubicin + Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Ondansetron	16 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	12 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Aprepitant	80 mg PO OD	Days 2-3 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	8 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.
- Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m^2.
- Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____